DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

Date: May 15, 2025

To: Office of Inspector General (OIG) U.S. Department of Health and Human Services (HHS) 200 Independence Avenue, S.W. Washington, D.C. 20201

Subject: Formal Complaint – Systemic Regulatory Non-Compliance, Policy Violations, and Governance Failures at Phoenix Indian Medical Center (PIMC)

To Whom It May Concern:

This memorandum serves as a formal complaint regarding the persistent and systemic violations of federal regulations, internal policies, and governing body oversight at the Phoenix Indian Medical Center (PIMC). The actions taken by the Chief Quality Officer (CQO), Clinton Pope, include repeated unauthorized policy changes, suppression of critical risk information, and failure to adhere to established governance procedures, resulting in significant legal exposure and potential financial liability for the institution. These violations align with the long-standing deficiencies previously identified by the U.S. Government Accountability Office (GAO) as part of the Indian Health Service (IHS) High-Risk designation.

Background of IHS High-Risk Status The Indian Health Service (IHS) has been repeatedly designated as a high-risk area by the GAO in biennial High-Risk Reports spanning more than a decade. These reports have consistently cited the following systemic deficiencies:

- **Inadequate Policy Implementation and Oversight:** Chronic failures to effectively implement and enforce policies, resulting in widespread gaps in patient care and regulatory compliance (2009, 2011, 2013, 2015, 2017, 2019, 2021, 2023, 2025).
- **Failure to Follow Federal Regulations and Internal Policies:** Widespread noncompliance with critical federal mandates, including Medicare Conditions of Participation, Indian Health Manual requirements, and Joint Commission standards (2009, 2011, 2015, 2017, 2019, 2021, 2023, 2025).
- **Poor Leadership and Accountability Structures:** Fragmented leadership and weak accountability frameworks that undermine effective oversight and operational consistency (2011, 2013, 2015, 2017, 2019, 2021, 2023, 2025).
- **Insufficient Internal Controls to Prevent Fraud, Waste, and Abuse:** Systemic weaknesses in financial oversight and internal controls, exposing the agency to significant financial and reputational risks (2009, 2013, 2017, 2019, 2021, 2023, 2025).
- **Deficient Quality of Care and Patient Safety Oversight:** Persistent gaps in quality assurance, patient safety protocols, and incident response systems, leading to preventable patient harm and regulatory sanctions (2011, 2013, 2017, 2019, 2021, 2023, 2025).

Documented Policy Overrides and Governance Failures

1. **Governing Body Bylaws (March 7, 2025, to April 24, 2025)**
    - **Incident:** Clinton Pope, Chief Quality Officer (CQO), executed multiple administrator overrides of the Governing Body Bylaws without proper committee review or formal governing body approval, including the following dates:
        - **February 5, 2025** – Administrator override by Pope, Clinton: Chief Quality Officer at 12:29PM EST.
        - **February 11, 2025** – Administrator override by Pope, Clinton: Chief Quality Officer at 12:03PM EST.
        - **April 9, 2025** – Administrator overrides by Pope, Clinton: Chief Quality Officer at 1:18PM EDT, 1:36PM EDT, and 1:44PM EDT.
        - **April 24, 2025** – Administrator override by Pope, Clinton: Chief Quality Officer at 11:30AM EDT.
    - **Regulatory Violations:**
        - 42 C.F.R. § 482.21 (Conditions of Participation for Hospitals): Requires governing body oversight for policy approval.
        - Indian Health Manual (IHM) Requirements: Policies must undergo formal review and approval by the appropriate governing authority.
        - Federal Tort Claims Act (FTCA) (28 U.S.C. §§ 1346(b), 2671-2680): Unauthorized policy changes that result in patient harm may expose the hospital to liability.
        - Joint Commission Accreditation Requirements: Failure to follow these protocols risks loss of accreditation and federal funding.
2. **Quality Management Policy (September 26, 2024, to December 2, 2024)**
    - **Incident:** Clinton Pope unilaterally approved the Quality Management Policy without the required full committee review, including multiple administrator overrides:
        - **September 30, 2024** – Administrator override by Pope, Clinton: Chief Quality Officer at 3:02PM EDT.
        - **October 21, 2024** – Administrator override by Pope, Clinton: Chief Quality Officer at 4:20PM EDT.
        - **December 2, 2024** – Administrator override by Pope, Clinton: Chief Quality Officer at 3:50PM EST.
    - **Regulatory Violations:**
        - 42 C.F.R. § 482.21: Requires governing body oversight of quality management policies.
        - Indian Health Manual Requirements: Mandates structured review and approval of quality policies.
        - Federal Tort Claims Act: Potential liability for adverse patient outcomes resulting from unvetted policy changes.
3. **Radiation Safety and Monitoring Policy (August 28, 2024, to October 10, 2024)**
    - **Incident:** Clinton Pope bypassed required departmental and executive reviews when issuing overrides on the Radiation Safety and Monitoring Policy:
        - **August 28, 2024** – Draft saved by Valdisera, Paula: Environmental Health Officer at 10:56AM EDT and 2:16PM EDT.
        - **October 10, 2024** – Draft saved by Pope, Clinton: Chief Quality Officer at 6:30PM EDT; Sent for re-approval at 6:31PM EDT.
    - **Regulatory Violations:**
        - OSHA Radiation Safety Standards: Requires formal risk assessments and policy oversight.

- Indian Health Manual Requirements: Radiation safety policies must be reviewed and approved through proper channels to ensure worker safety.
- Joint Commission Standards: Noncompliance with safety standards risks loss of accreditation and increased liability exposure.

Conclusion and Request for Immediate Review Given the persistent pattern of policy overrides, noncompliance with federal regulations, and the suppression of critical risk data, I respectfully request that the OIG conduct a comprehensive investigation into these ongoing issues at PIMC. These actions represent a continued failure to address the very deficiencies that have kept the IHS on the GAO High-Risk List for over a decade, posing significant risks to patient safety, regulatory compliance, and the appropriate use of federal healthcare funds.

Thank you for your attention to this urgent matter. I am available to provide additional information or documentation as needed.

Sincerely,

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Attachments: Supporting documentation (available upon request)